



Dolores Jean Lavins Center for
Humanities in Medicine

The Tempest

Mayo Clinic Alix School of Medicine
Annual Creative Arts Publication
2022





Dedicated to
Johanna Rian

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Foreword

Welcome to the 2022 edition of *The Tempest*: a collection of original poetry, prose, visual art, musical composition and photography by Mayo Clinic graduate students. You will encounter themes of tragedy, self-doubt, exhaustion, recovery, discrimination, cultural identity, communion, solace in art and nature, nostalgia and forgiveness.

The title of this publication is rich in metaphor. It was inspired in part by the foundation of Saint Marys Hospital in the wake of three tornados that swept through southeast Minnesota in August 1883, killing more than 30 and injuring

more than 200 people. With no local hospitals available, the injured were cared for in Rommell's Dance Hall, which stood on the corner of today's First Street and South Broadway, and the Convent of the Sisters of Saint Francis.

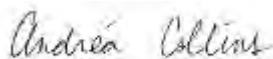
The title was also inspired by Shakespeare's *The Tempest* (1611), a play that opens with a shipwrecking storm. The audience learns in the following scene that the storm was conjured through magic by the vengeful former duke of Milan and an airy spirit. A young prince named Ferdinand and his father believe that one another drowned.

When father and son are reunited in the final act, Ferdinand rejoices: "Though the seas threaten, they are merciful. I have cursed them without cause."

As I near the end of medical school, Ferdinand's relief reminds me to continue to brave the storms around us, to recognize the pain and beauty of the journey, and to hold on to hope.

May the talent and wisdom contained in this edition bring you light.

With love,



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Introduction

I have had the privilege of learning to know many of the students whose work is published in this eighth edition of *The Tempest*. When recently asked what I would consider to be highlight of my career, I answered without hesitation: working with Mayo medical students--in the classroom, in committees, in producing creative works such as this journal. Without exception, I have found them to be unfailingly kind, compassionate, perceptive, and committed to making a difference in the field of medicine.

The work published here offers a composite portrait of these extraordinary students. Often describing or evoking a swirl of contradictory emotions--Sherry specifies composure, self-doubt, happiness, paranoia, resilience--the poems, photographs, drawings, and paintings speak to the push/pull that characterizes the learning trajectory

of medical education. Despite the acknowledgements of struggle, fear, or despair evidenced by Neha working three jobs in college, the breathtaking disrespect of Alyssa by a team member, and Rachel's grief for the patient she could not help, again and again these authors/artists report the realization that every opportunity offers education and truth resides all around us. Yong-Hun sees the full dimension of his grandfather's life in a coming-home handshake, Mihai embraces the choices inherent in the many crossroads before him, and Jessica's plea rises to become a claim to dignity. Others speak of insights gained at the moment of crossing from the particular to the universal or from the immediate to the infinite, that favored moment of poets that encapsulates wisdom. Luqman's evolution from self-absorption to compassionate humility, James' Half Moon Bay sunset that inspires persistence, and Gordon's

recognition of the unique person embodied by his first patient, "And on his hands he tells the stories of his life," all provide lessons in redemption and enlightenment for the reader. Occasional ironies appear, as in Aisha's silent image of the fallen tree and Jeff's tribute to the mentor to whom he may have been the last new friend. In my view, one of the more joyful evocations of these discoveries is Santiago's piano composition. He says that the piece connects all past and future versions of himself so that when he plays it, "it's like a party." Santiago, his fellow contributors, and Editor-in-Chief Andrea Collins in her forward remind us to "hold on to hope." I am grateful for the message, and for the opportunity to introduce this remarkable collection of creative works.

Johanna Rian, Ph.D.

Program Director, Dolores Jean Lavins Center for Humanities in Medicine

It is truly a privilege to help introduce this eighth edition of *The Tempest*, authored and produced by our Alix Mayo School of Medicine student colleagues. Each edition has proven to be a remarkable collection of artistic and literary contributions that reflect the values and creative capacity by a group of gifted individuals whose expertise in the sciences is clear. Each submission in its own way represents reflective thought, imagination, and a sense of presence -- an object that calls on us to stop, look, listen and reflect if even for a brief moment in a world now functioning within an attention economy with multiple priorities demanding our focus at any given point in time. Now more than ever the creativity and imagination

demonstrated herein continues to empower our ability to move beyond the distractions of the present and into a vision of the future marked by hope and a commitment to 'the other' writ large.

I would also like to take this opportunity to thank Dr. Johanna Rian for her tireless devotion to the Mayo Clinic Dolores Jean Lavins Center for the Humanities in Medicine at Mayo and to her collegial work in the humanities throughout the Mayo Clinic enterprise. Through these efforts Dr. Rian created and introduced a formal Mayo Medical School Humanities Curriculum, established a Distinction Track for students making their humanities education a priority, and helped to lay the foundations for a

Certificate Program in humanities. She is a nationally and internationally known expert in our field whose influence in establishing a model for humanities teaching nationally and internationally will prove to be instrumental in medical school education in the coming years. We will miss our daily dose of good humor, creativity, curiosity, imagination and leadership, and look forward to her future involvement in individual project leadership.

Daniel Hall-Flavin, M.D., M.Sc.

Medical Director, Dolores Jean Lavins Center for Humanities in Medicine

Grandfather

BY YONG-HUN KIM

Grandfather.
“Welcome home.”
He wears my college sweater.
A firm handshake,
Pride begetting warmth.

Comrade.
Of his courage they joked,
“He who cannot see
through narrow eyes
charges blindly ahead.”

Father.
During war,
He walked away
At the height of his career
To keep his family safe.

But that night,
When lights and smiling faces
Greet me inside the flat,
His hand feels
Colder than mine.

The eyes that shielded
Him from the unknown
Now betray his fear.

His family,
Framed in his heart,
He could not leave.
A prolonging death—his final sacrifice.

River at Dusk

BY EMILY STICKNEY

Acrylic on Canvas



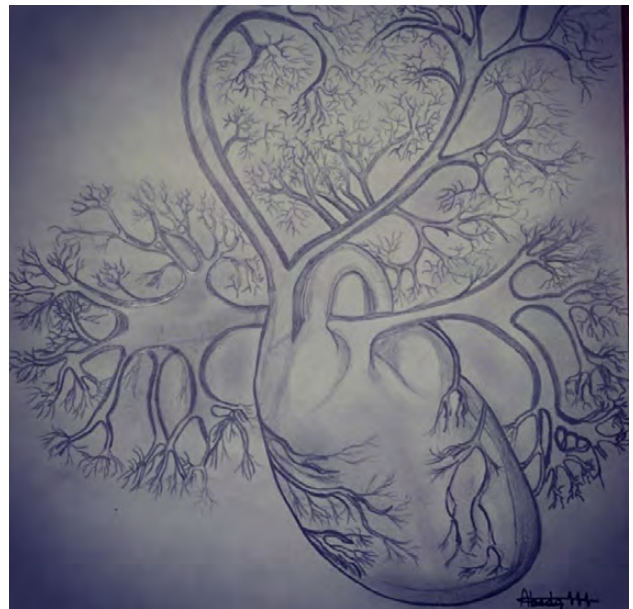
Woman of Africa

BY GRACE PATRICE ANYETEI-ANUM



A Heart Full of Love

BY ABDUL KARIM GHAITH



Synaisthima

BY SHERRY ZHOU

Composure is a colorful chameleon
lounging on its favorite branches
Mingling among the shrubbery from
which it borrows protection Its gentle
yet sure grasp evenly supporting its
unseen weight.

Self-doubt is a famished hyena
desperately searching for a carcass Its
sharp, bone-breaking bite tears savagely
into its meal Only the lion can give it
pause over its precious prize.

Happiness is a chirping bird whose
presence comes and goes Much motion
can disturb it from its transitory perch
A bark, a rustle, a light gust sends it
soaring out of reach.

Paranoia is a mega-murder of crows
suffocating the sky in a swath of black
Their taunting and cackling caws
echoing over and over endlessly Every
wing flap, every croaking call creating a
constricting cacophony.

Resilience is a spawning salmon
struggling upstream to its birthplace
Doggedly, it swims, jumping through
Gaping maws and over slippery rocks
In the quiet, trickling waters it finds
fulfillment, certitude, renewal.

North Shore

BY MICAELA WITTE



Whoever Saves a Life is As if They Saved All of Humanity

BY REWAN ABDELWAHAB



On the shoulders of normal-sized giants

BY JESSICA WENG

Bleary-eyed and surgical cap awry
I follow
In single file line; it's nearly nine (PM)
And I wonder
How do these giants still walk with urgency, determination, purpose?
How do they still smile at the worried families with crinkles at the corner of their eyes?
And I think
About deft fingers and strong arms
As they stand for hours long
To care for the women whose bodies have done them wrong
Exhausted, I fade
The blue of scrubs and the white of coat become a new shade
A shade that is power and beauty
A new blue that sustains on or off-duty
The lines blur and seemingly expand
How was I in the presence of people so grand?
Suddenly, I realize
That the giants in front of me
Are not a fantasy
They are normal-sized people
Giant in all they do and speak and serve
From all that I observe
So how do I
Stand on the shoulders of these giants?

Sunset

BY JAMES HWANG

New Year's Eve on the West Coast
It's cold and windy, but serene

I capture the moment by photograph,
My relic of Half Moon Bay

Where a pool of fresh water lies
A remnant of last night's storms

And a couple sits on the sand
Enjoying the daily spectacle Nature puts on

Looking forward with hope
Despite the obstacles of the past year

Just like the previous year
And the year before that

Despite the uncertainty, the trepidation
An unknown future brings

We welcome the darkness of dusk
With a persistent, almost stubborn naïveté

It's nightfall we face
But it's a new beginning we feel

We are hopeful, strong and
We persist.

Half Moon Bay State Beach, New Year's Eve 2021

BY JAMES HWANG



Maroon Bells

BY KACEY KEMPER



MD Doubts

BY NEHA DEO

*You can't do medicine, you're not smart enough,
So I sacrificed the weekend nights, choosing to review my notes.*

*You can't do medicine, you're not wealthy enough,
So I worked three jobs at a time, so I could afford the bill.*

*You can't do medicine, you're not patient enough,
So I kept myself busy, and found ways to enjoy the years.*

*You can't do medicine, you're not strong-willed enough,
So I found my people, those who I could lean on when I felt low.*

*You can't do medicine, you're not good enough,
So I told them, I'll be worthy of the MD soon.*



Head and Neck Anatomy

BY ALYSSA BROWN



When I Close My Eyes

BY GRACE CHANEY



Cultural Humility in Healthcare

BY EWOMA OGBAUDU

Long before I was a medical student and burning the midnight oil to make sure I knew all of the branches of the facial nerve, I spent several years honing my craft as a photographer. I discovered the art of photography during my freshman year of college somewhat serendipitously. My friend allowed me to use his Canon DSLR camera one day while we were walking through Morningside Park on the Upper West Side of New York City, and I was blown away by the details in the world around me that I never noticed before. Photography, in a way, was my first love. What excited me the most about this was that photography transformed me into a visual storyteller and attuned me to the backgrounds and cultures of the people around me. I joined the Columbia University Photographers' Association and used my camera to document the lived experiences of people I encountered on campus. Listening intently during our conversations, I learned about how Dominicans ate plantains with countless meals like my Nigerian family and I also picked up on the nuances that were unique to our backgrounds. These experiences gave me a strong appreciation of other cultures and helped me understand that people do not exist in isolation but rather within complex social, cultural, and economic environments. It is the interplay of these factors that not only influence one's identity but also one's health. Thus, doctors should aim to develop a patient care model that is based on cultural humility in order to best serve their patients.

Healthcare grounded in cultural humility, rather than cultural competence, is other-oriented and recognizes that we can never become experts on every dialect, custom, and belief of another culture. This form of healthcare is dynamic and encourages medical professionals to engage in lifelong learning about other cultures and self-reflection to provide care while upholding patient agency in their course of treatment. As a first-generation Nigerian immigrant, I learned to adapt the principles of culturally sensitive care while taking care of my mother after she tested positive for COVID several months ago. Despite my pleas for her to go to a hospital, she insisted that she would simply make agbo, a traditional herbal beverage, to cleanse herself of the infection. Recognizing that she wouldn't budge without this treatment, I brokered an agreement wherein she would drink the agbo and later go to the hospital so that she would get the best of both worlds. Since then, I have since found other ways to compromise with my parents while encouraging them to make positive lifestyle changes. One of the ways I did this was by purchasing an air fryer for them so that they could still enjoy the crispy plantains that they grew up eating while only using a fraction of the oil needed.

Additionally, we fail to uphold the ultimate aim of culturally humble healthcare if we do not use its principles to address health disparities. While taking a photo-walk through the juxtaposed neighborhoods of the Upper West Side and East Harlem in New York City years ago, I was shocked as the finely manicured grass faded into rusted playgrounds held together by duct tape and prayers. The tangible difference in living conditions and a mirrored drop in life expectancy exposed gaps in healthcare that weigh on my heart till this day. Adopting a healthcare mindset that is based on cultural humility opens our eyes to these social determinants and allows us to provide focused care. To truly be stewards of our patients, we must place their needs and those of their respective communities at the forefront of our minds by devoting ourselves to achieving health equity through cultural humility.

Photography and caring for patients are not mutually exclusive but rather they are both artful skills that need to continuously be practiced. While I was an undergraduate student, my camera was a tool that gave me insight into the cultures of others and helped me learn about their stories. In turn, healthcare built on cultural humility is a tool that helps us rewrite the stories of our patients and communities by addressing systemic barriers and equipping them with the agency to reshape their narratives.

Restless

BY GORDON XIE



Dreaming of Monument Valley

BY GORDON XIE



The Ladder

BY PAIGE CUMMINGS

What is privilege if not preceded by
white?
It's standing on rooftops but still having
sight
Of the people beside you holding the
ladder upright
They stared from the ground to the roof
I stare from the roof to the sky
They stood together in segregation
Wondering how high they'd go
If they all kept on climbing
I stand alone in isolation
Wondering how far I'll fall
If the ceiling keeps rising
I'm taking up space
On a rooftop that's bending with weight
I feel out of place
The privilege I hold
It's a story untold
I stand on the backs of histories sold
Surrounded by those by which history's
told
With all that I've learned
I fear of privilege unearned
The people beside me

They didn't invite me
They remind me
I'm taking up space
On a rooftop that bends under weight
They push me out to the edge
My toes on the ledge
I'm so close to the end
I bend
I see what they cannot comprehend
Just within sight
The people below me holding the ladder
upright
Then I know my place in the fight
To give it more height
To aim from the roof to the sky
If I fall from up high
I'll crash on a rooftop that bends under
weight
Maybe it breaks
We'll all learn how to fall
Then I'll build a new house with no
ceiling at all

Blue Skies of My Past and Future?

BY SAM BUCHL



a warm throne
for the royal kitten —
power adapter

BY SHERRY ZHOU

My Third Degree

BY RICHARD EBOKA

Burn!
Intubate –
Airways churn,
Noxious vape!
Calm down. Inhale. No fumes...

Alimentary feeds,
Karmic goodwill
Auspicious indeed –
Nares, oxygen-filled
Gravid night air, gratitude!

The Size of My Scrubs

BY ALYSSA BROWN

Blue, white, red, yellow, pink, and brown. These are the colors of the ties and strip of fabric around the scrub pants and scrub tops. Maybe it sounds dramatic, but it always felt like a scarlet letter to me. The colors indicate the size of the scrubs. My friends always complained about not being able to find smalls and mediums, and I would shrink back a little more. At the start of medical school, I would squeeze into the red top and red pants. These were the larges. It was so easy to see who was smaller or larger than you because of this. I felt uncomfortable because I knew how many other people were smaller than me. I was worried they were judging this too. I had spent years growing up uncomfortable. The southern term for it is being big boned, and everyone in my family is big boned. On the golf team in college, I was used to having to order the biggest size on the team, and it made me uncomfortable. It was slightly illogical because I was also the tallest and strongest on the team, but it was ingrained in me to feel uncomfortable with my size.

Maybe I overthink it, and no one else cares what size of scrubs I am wearing, but I noticed. By the end of third year, I started wearing yellow bottom scrubs (XL) and red tops. The irregular eating in the hospital led me to increase in size. Also, I was tired of squeezing myself into the smaller bottoms. My friends did not have this problem. They often lost weight through medical school. They ate less and less because of stress, while I ate more.

During my third year, I started feeling more and more uncomfortable with comments that people made to patients and to me about weight. I can't count the times that people commented on how the patients should just lose weight and how they could just eat less. They would call them obese or fatties, but I knew how much they weighed and knew it was close to my own weight. I was acutely aware of the struggles of trying to lose weight. Sometimes it is not as easy or simple as just losing weight.

One situation stuck out to me. I was stretching my calves after a long case, and I asked the other student with me if his calves were stiff too. He responded that mine were probably just sore because I was fat. I didn't know what to say. I just turned around and left the operating room. I never said anything to him about it. I just ignored that he said it. I shouldn't have. It wasn't fair for him to say it. My calves were stiff because of the long operation, not because of my weight. It confirmed my fears that everyone was judging me for my size.

When I came back for fourth year, I moved into pink top scrubs and pink bottoms (XXL). I knew I had gained weight, and this was just another reminder. I have worried on the interview trail this year about being judged for being bigger. I don't often see people that are my size. I often see the same thinner students. I worry that it is an unspoken thing that I will be judged on, which is supported by data in studies of graduate school admissions¹ and radiology interview invites.² I worry the people interviewing me will view me as lacking discipline for being overweight or that I am unfit to be a surgeon because of it. Even though I am still strong. I still get around the hospital just fine. I still take care of the patients like everyone else, but I do not want to be judged for this appearance, even though I know I will be inevitably.

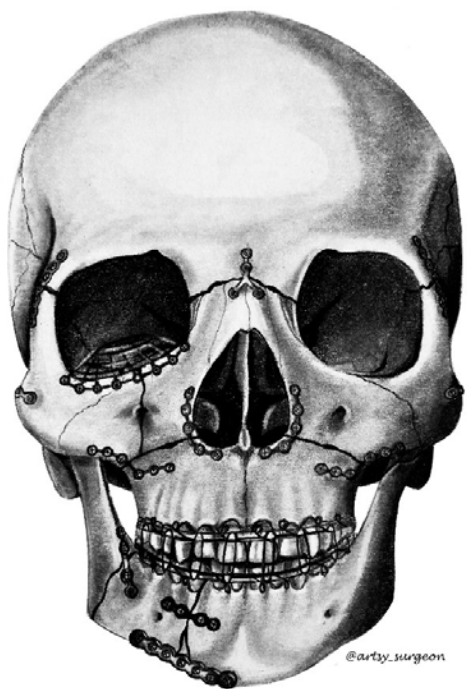
I'll try to focus less on the color of scrubs I fit into. I realize they are just scrubs, and their size does not indicate anything about who I am as a person or eventual surgeon.

References:

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Minding My Own Business

BY YULIYA PETUKHOVA



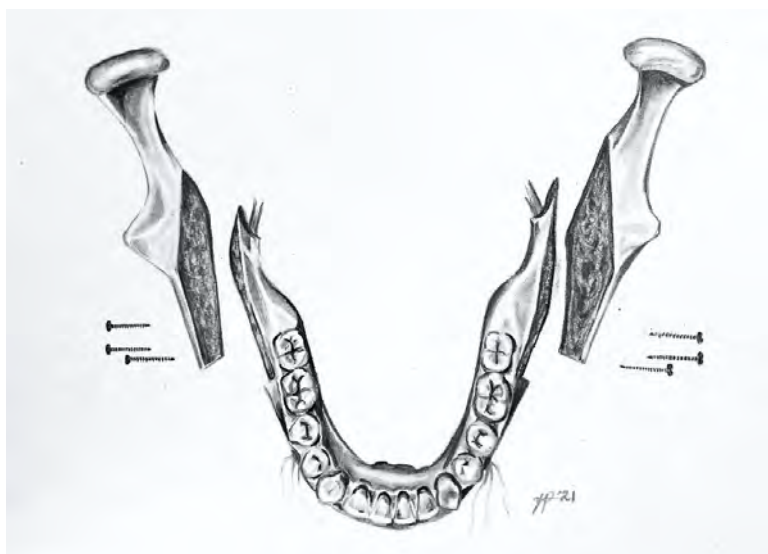
TMJ Total Joint Replacement

BY YULIYA PETUKHOVA



The Splits

BY YULIYA PETUKHOVA



Of Steel and Formalin

BY GORDON XIE

At first we
wondered at the facts they laid before us,

But hard-earned
knowledge keeps the wonder now at bay,

As if an
Innocence inherent in unknowing

Has been
resected in that sterile day-to-day,

But when we
look into those eyes no longer seeing

And sink our
blades into the skin that can't feel pain,

We still
perceive beyond the flesh another being;

We feel a
presence that our facts cannot contain

Of someone
greater than the sum of all his pieces,

Whose inward
truth will never part beneath the knife,

For on his
face he wears a dream within his creases,

And on his hands
he tells the stories of his life.

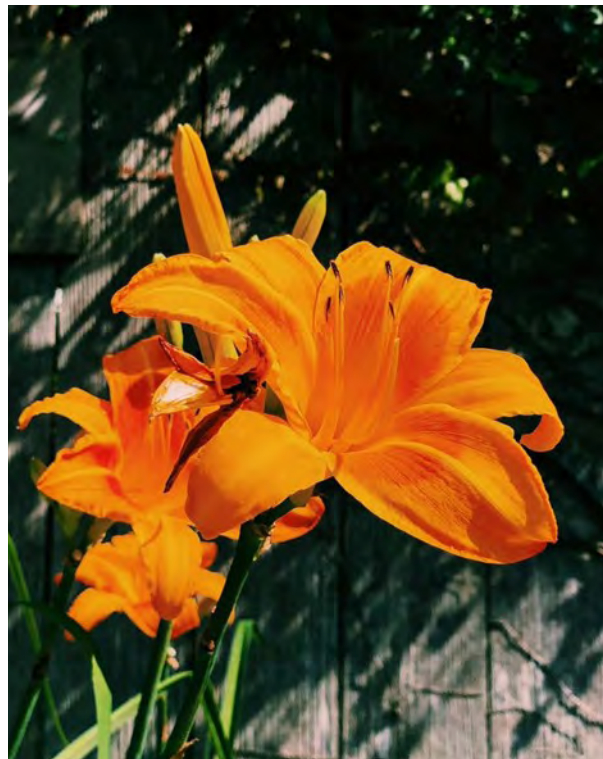
Bent Spoon

BY REWAN ABDELWAHAB



7/25

BY AISHA ADEN



My “First Patient”

BY JEFF WOODS

Like many Mayo medical students, I watched *The First Patient*, a 2018 documentary following first-year Mayo medical students through the process of dissecting a cadaver, during my first-year anatomy class. Then (and still now), I found it borderline absurd to hear a cadaver spoken of as one’s first patient. I went into medicine to treat patients biomedically, yes, but always in relationship to their humanity, their wishes, fears, goals, and ways of finding meaning. Without that, how, I asked myself, could a cadaver possibly be my first patient?

Fortunately, as I was starting anatomy and asking myself that question, I had the pleasure of meeting and speaking at length with Rev. O, an elderly man I was looking forward to following longitudinally and learning from for the subsequent four years of medical school. In what turned out to be our short time together, he inspired me and taught me more than any other patient with whom I’ve interacted. Dedicated to him, this is the story of my “first patient.”

Rev. O, a 98-year-old veteran and former Lutheran pastor, was born in Minnesota in 1922 and joined the army to serve in World War II in 1942. During the war he served as a forward observer, a designation that involved dangerous missions frequently operating behind enemy lines to direct artillery strikes on enemy positions. He spoke with enthusiasm and pride about his time in

the military, a time that sounded full of both great adventure and momentous purpose. Unfortunately, he was injured in combat after serving for almost three years, and his injury caused substantial permanent hearing loss.

We spoke on Facetime rather than the phone because Rev. O relied heavily on lip reading to communicate. Hearing his life story made me aware of and challenged a preconception I had about age and sensory loss: I had assumed he had difficulty hearing because of his age, but, of course, I soon learned he had lost his hearing three-quarters of a century earlier in the war. Even that relatively innocent misapprehension of mine gave me pause—it was a small manifestation of my own ageism, a way I made an assumption about Rev. O based on his age only, without knowledge of his unique life story and history.

We briefly discussed the struggles Rev. O’s hearing loss had caused him throughout his life and in his old age, but it was abundantly clear that he let very little bother him. No matter what topic we discussed, he spoke of the immense gratitude he felt for how blessed his life had been. He appeared to harbor little or no resentment, in spite of significant losses over the course of his life, including his war injury, the sudden death of his wife more than 30 years prior, and the death of his daughter in 2019, at the age of 70.

There was some sadness in his voice, but perhaps more weariness than sadness, when we talked about his wife’s death thirty years ago. I could scarcely imagine how it would feel to endure the loss of a spouse for so long, but after reflecting, I realized the weariness he seemed to feel made sense—weariness from carrying the weight of grief and loss for so long. He said he’d never dreamed he’d live this long, outliving his wife by decades and even outliving one of his daughters. When we talked about the death of his daughter, he didn’t seem very sad; he said she lived to an old age and had lived a good and full life, so there wasn’t much to be sad about. For myself, I imagined it would be very sad to lose one’s child no matter the age, but Rev. O’s peace at her passing made me reconsider.

We talked about the challenges of getting older, and he said the most difficult aspect of aging was losing his friends and peers, nearly all of whom he’d outlived. Rev. O had actually retired as a Lutheran pastor in 1987 before losing his wife two years later and deciding to resume working. He subsequently worked as a part-time pastor at Bethel Lutheran Church in Rochester, where he ended up working for over 20 years until re-retiring in 2009. He remarked that, having already lived an additional 11 years after his re-retirement, perhaps his second retirement was as premature as his first.

I imagined working until the age of 87 at a job oriented around meaningful connections with others must have brought richness and meaning into his life. He also had a very large family, including two surviving kids, ten grandchildren, and more than 20 great-grandchildren. Yet, he still felt loneliness at outliving so many friends and peers and a disbelief about his own longevity. I wondered, is it inevitable to feel that loneliness as one grows old? We fall so far short of caring well for our elders in American society, but even if we did much better--and Rev. O's old age seemed much better than most elderly individuals I've known--is loneliness inevitable in old age?

Rev. O was the most cognitively sharp 90+-year-old I have ever had the pleasure of meeting. We spoke a lot about his career as a Lutheran pastor as well as my own experiences studying theology and also growing up Lutheran. Communication was challenging in spite of talking on Facetime; Rev. O seemed to understand less than half of what I said. During our conversation, I pivoted to speaking very little, instead just listening to his incredible life story. I sent a follow-up email after we got off Facetime, reiterating some of the things it seemed he hadn't been able to understand. I wondered how we could communicate more effectively going forward. I vowed to speak more slowly and to enunciate more clearly in our next conversation.

I was flattered that Rev. O invited me to accompany him to his next routine doctor's appointment at Mayo Clinic, together with his brother, who regularly accompanied him to his appointments to make sure he didn't miss anything the doctor told him. Aside from his hearing issues, he said he didn't have any other chronic health conditions worth mentioning; although, I wondered if he just preferred to speak of things from his life that were more richly meaningful and important, as opposed to the mundane-by-comparison details of his health. He spoke very well of his doctors. He relied on his brother to relay details of his conversations with doctors because of his difficulty hearing.

In addition to Facetime, Rev. O relied on email for communication in general. He was quite tech-savvy. In fact, because I don't own any Apple devices, I had to borrow my wife's iPhone to Facetime with him, so he ended up teaching me how to use Facetime. His facility with Facetime challenged another preconception of mine about aging--being in one's 90s does not preclude one from mastering new things, including new technologies.

After I didn't receive a reply to multiple emails I sent Rev. O in October, I started to fear the worst, since he'd always quickly respond. Indeed, he didn't reply because he had passed away, on October 9, 2020. I felt such an uncommon warmth for and connection to Rev. O, and I was deeply affected by his death. I felt as though Rev. O had been my first patient--and sadly the first patient I'd lost.

I found myself wishing I'd done things differently. I wished I'd pressed him more to share the details of his health condition, as if I could have done something to help. I wished I'd arranged our second conversation sooner, rather than being too busy with anatomy. I wished I'd put more effort into figuring out how we could better communicate in real time in spite of his hearing difficulties. I thought we should have tried some voice dictation software so he could have read my words as text in real time as we talked, and I wondered why I hadn't thought of that sooner. Did I think it was alright for him to not understand me as we spoke because that was just part of getting old? Was that ageism at work again? Did others in his life (including his doctors, who he struggled to hear) think the same way I had? Would he have felt less lonely and would he have even lived longer if I--and others--hadn't been so ready to accept that we couldn't communicate well because of his hearing loss?

Rev. O inspired me in the brief time I knew him. He helped me appreciate the incredible wealth of experiences every older person I meet brings, the weariness and loneliness that come with getting older, and the ways that perhaps some creative thinking and a desire to connect might make a difference. I feel grateful and honored to have been able to speak with Rev. O and very grateful to have been perhaps the last new friend he made.

Crystal Bay, Silver Beach, MN

BY ANJALI PANICKER



Something interrupts my quiet reverie

BY ANJALI PANICKER



Spot the Mountaineer – Palisade Head

BY AISHA ADEN



Team

BY RACHEL LOPDRUP

To my patient I couldn't help,

I saw your family give you their last goodbyes,
Hoping your ruptured aortic aneurysm was lies.
Hoping and wanting for you to survive. And in the corner,

I couldn't help you.

To your daughter and wife, you breathed, "I'm sorry."
Sorry for what? I may never know. I'm hardly
Supposed to be part of this conversation. I'm sorry

That I can't help you.

I wish we could have met under better circumstances
Like in the clinic where I will see your nieces and
Take care of their emotions and hypertension

But I can't help you.

If you were but younger, if I'd known you earlier,
If you and your family were part of my panel or
If you'd seen primary and evaded this overture

Could I have helped you?

I wondered what story you hoped for your rest,
If you were open to discussing death,
But unfortunately that's not we do in the west

So I couldn't help you.

Your family touched me, and I am so privileged
To have seen a part of you not one else had witnessed
And I look forward to being there with you

If it happens again, because our team did help you.

fall-ter

BY SHERRY ZHOU



Did it make a sound?

BY AISHA ADEN



Ascent

BY EMILY STICKNEY

Acrylic on Canvas

Dedicated to Bran Ghislain



Where the Heart Is

BY EMILY STICKNEY

Pen and Watercolor on Paper, Polymer Clay Earrings



There's More to Life than Milestones

BY ANJALI PANICKER



Republic Reflections

BY ELLIANA HUFFMAN



Blooming Dreams

BY ELLIANA HUFFMAN



I AM POEM

BY EWOMA OGBAUDU

I am from **MUSIC** like...

- Afrobeats that reverberates through my bones and generates rhythmic manifestations of my internal joy

I am from **SOUNDS** like...

- Pentecostal prayers and incessant calls on my family's house phone from bill collectors

I am from **PLACES** like...

- the Olomoro village in Nigeria where both of my parents were born and where my father tilled the land of his ancestors as a farmer
- Surprise, AZ where the summer heat melts ice cream and inhibitions

I am from **SAYINGS** like...

- "We will definitely not shut up and dribble"
- "You have to work twice as hard to make up for the privilege you were not born with"

I am from **PEOPLE** like...

- Abraham, Isaac, and Jacob as I am a child of God

I am from **JOBS** like...

- Frying chicken as a KFC employee which was both of my parents' first jobs in America

I am from **LOVE** like

- "Have you eaten yet?"
- The smell of cocoa butter on my mother's skin as she pulls me in close for a warm embrace

Under the starlight, we'll find our way

BY ABDUL KARIM GHAITH



Lincoln Lights

BY ELLIANA HUFFMAN



All roads lead to the kitchen, don't they?

BY ANJALI PANICKER

"we believe good food brings us together," it read on the brown paper bag from the People's Food Co-op at Rochester, MN.

This concept has made a massive impact on my life and has contributed to my integration into a new country and culture.

It has been 5 years, but it feels like yesterday when the medical biotechnology class in Rockford, IL, would get together for potlucks, cookouts, barbeques, and festivals. To give you an idea of the demographics, we were 34 students: 26 Indians, 5 Americans, and 3 Saudi Arabians. With so many classmates being Indian, I knew it would take effort, curiosity, and *going out of the way* to learn about the new place, people, and culture. Holding on to the safety of familiarity was not why I had relocated 13000km (~8000mi) away. To say that the program staff made this goal of integration easy would be an understatement. I will forever be indebted to SH, MW, NO, and LS for their efforts and initiatives. They organized a plethora of events and activities with food at the center of each. These get-togethers were the reason:

- I understood what a Superbowl was. (I knew nothing about American football)
- I celebrated the Cubs winning the World Series in 2016. (I did not know much about baseball either – It only looked familiar because of cricket)

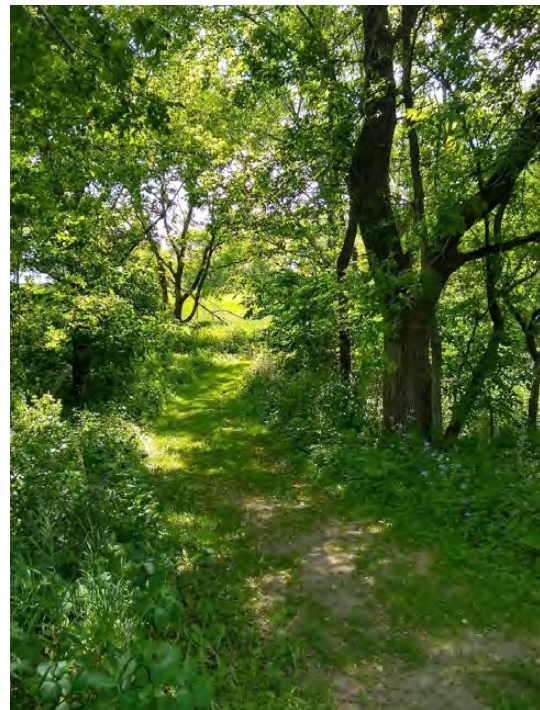
- I feasted on turkey for Thanksgiving. (It was not common where I came from – I realized what I had been missing)
- I tried Bacon-Wrapped Walnut-Stuffed Dates with honey-balsamic reduction (Earlier I could not fathom meat with sweet – I have changed my mind without any resistance)

I am also grateful to my parents and grandparents, not only for the values but also for the recipes they have shared with me. Those have won many hearts and gifted me otherwise unlikely friendships along the way.

2017 Easter Sunday Menu:

- Appetizer: Indian Fish Fry (different from its Midwestern counterpart)
- Main course: Chicken Biryani with a side of raita
- Dessert: Gulab Jamun
- Little did I know the aroma of these spice blends roasting would lead to a new friendship.

Mrs. D was our then-68-year-old Caucasian neighbor. I typically waved or greeted her if I saw her working in her yard when I walked to the university. All I knew about her was that she was a substitute teacher at the Rockford School District – information I registered in one of our fleeting conversations. That Easter Sunday, the fragrant spices from our kitchen piqued



her curiosity - she stopped by. Our conversation led to a few preliminary food exchanges. She then invited me to join her when she walked G, her bulldog-cocker spaniel, around the block or by the Rock River. I got to know her better during these regular evening walks. Our conversations comprised recipes, nutrition, health, my research, her kids and grandkids, privilege, immunology, racism, pedagogy, my family back home, and everything in between. She even gave me her old bike that I fixed and used for grocery shopping. That summer, we plucked fresh raspberries from her backyard, and she taught me her recipe for raspberry bread.

Mrs. D knew I worked long hours in the lab. I did not get home before 1 am on most weeknights. I would return tired and hungry, unenthusiastic to cook. I cannot count the number of times I found a container of delicious wild rice soup, cauliflower casserole, or another hearty meal waiting on my porch. On this container, I would find a sticky note with words of encouragement in Mrs. D's beautiful handwriting. I like to believe it was her kindness that helped me through many failed experiments during my MS. After I wolfed down the delicious sustenance, I would be left with her empty containers.

A simple rule I was taught as a kid:
You never return a container empty. If someone puts in the time, effort, and thought to share something with you, you express your gratitude by sharing something thoughtful with them. A novelty I added to this rule: *Let it be unique and handmade, not store-bought.*

So, on the weekends, when I was away from the lab, I would cook something scrumptious to return in her containers. That exchange was also an excuse to meet my friend, go on another walk, and find out which kid from her class was a *smart cookie* that week. The year I spent with Mrs. D was the highlight of my time outside of grad school. To date, we make it a point to call each other, send Christmas and birthday cards, or even meet up in good old Rockford, IL.

The strongest of bonds, most meaningful of friendships, and most unexpected yet cherished encounters in my life have stemmed from the exchange of food. Food is the reason two strangers with different upbringings, ideologies, and experiences now treat each other like family. I have experienced such friendships blossom over food even after moving to Rochester, MN. AO, JG, MK, YW, JG, MG... the list goes on. Apart from the lovely family I found in these people away from home, food has also offered the opportunity to learn a few virtues:

Thoughtfulness – The last thing I wanted to do was scare people away from the food I am so passionate about. Indian cuisine tends to have a reputation for being spicy. Chilies and peppers make it hot, but other whole spices make it flavorful, I would argue in defense. It took a conscious effort to be cognizant and considerate of the taste buds in my circle of international friends.

Inclusiveness – Potlucks teach us to be cultured and accommodating, cultivating acceptance over mere tolerance. I found it was the impetus for culinary creativity - substitution genius, if you may. I challenged myself to create dishes with ingredients from the overlap in the Venn diagrams compatible with the various dietary choices.

Humility – When someone asks you questions about the ingredients, the technique, the procedure, or the equipment you used, you will realize how little you understand about your own cuisine. There is always more to learn...

... and I look forward to new culinary adventures. All roads lead to the kitchen, don't they?

Autumn Day

BY ABDUL KARIM GHAITH



Summer Nights

BY MICAELA WITTE



A Plea

BY JESSICA WENG

I am not my black hair, or slanted eyes, or high cheekbones.

Though I do love them.

I am not the other powerful and beautiful Asian women in my class.

And I do love them.

I am not “that other Asian girl”

Though I do love my heritage.

I am not invisible.

Though sometimes I feel their eyes glaze over me to someone they recognize as their own.

I do not want to be pleasantly surprised when someone calls me by my own name and not by another Asian woman’s name.

Though I rarely have that pleasure even still.

I do not want to feel discouraged when yet another study shows that Asian women aren’t perceived to be leaders,

And when I think about the Asian women who are still fetishized, abused, and raped

In the theater of war and the theater of entertainment

Under the lie that we are submissive.

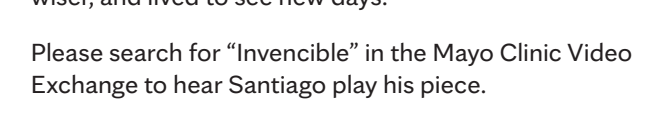
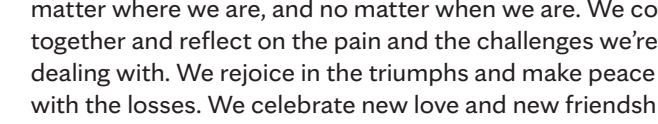
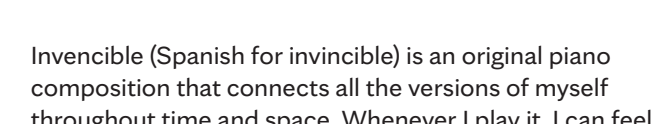
But we are not.

See me. Recognize me. Know my name.

Invencible

BY SANTIAGO RESTREPO-CASTILLO

The musical score for "Invencible" is presented in two columns, each containing four systems of music. Each system consists of a treble staff and a bass staff. The music is written in 4/4 time and features a consistent eighth-note bass line in the left hand. The right hand plays a series of chords and intervals, including octaves and dyads, creating a rhythmic and harmonic texture. The piece concludes with a final chord in the right hand and a sustained bass line in the left hand.



Invencible (Spanish for invincible) is an original piano composition that connects all the versions of myself throughout time and space. Whenever I play it, I can feel my past and future selves playing it too. Truly, whenever we play it, it's like a party, because we all come together, no matter where we are, and no matter when we are. We come together and reflect on the pain and the challenges we're dealing with. We rejoice in the triumphs and make peace with the losses. We celebrate new love and new friendships. We remind ourselves that we're invincible, that we've fought countless battles, and that we've always prevailed, grown wiser, and lived to see new days.

Please search for "Invencible" in the Mayo Clinic Video Exchange to hear Santiago play his piece.

churches I have been to

BY TORI RICCELLI

i've been to church
on a dark cold night in january

when woolly city light smudges the sharp edge of darkness like an artist with
graphite-dyed palms

where a blanket of black presses in on old gas lamps sparking with electricity
and ghosts murmur quietly under shimmering pools of golden light

with sounds of oxford shoes on old medieval stones
and the sharp tang of love and youth in the air

surprise that love smells like wet earth and stone tombs

i've been to a church that has been here hundreds of years, maybe thousands

its smoking candles holding up the silent, holy night
metal workings and altars flickering pinpricks of light in a stark northern sky

and the time travelers sing the songs of their ancestors, for here God is truly the Ancient
of Days- as ancient as the church and even farther

a still small voice in the dust of bones and saints

i've been to church
on a bright warm morning in may

when sun slips silky through open window shades
swirling invisible house currents of oxygen and dust

where the morning is quiet, but for the padding of cloth slippers
and the scrape of a spatula on a garish pink bowl

the ssssz of batter. the *plop* of berries
hazy sugar-scented dreams under tables play hide-and-seek

i've been to a church on a computer, here only for the hour

a southern preacher in a speaker with background pops of technicolor
dipping archaic words in schmaltzy, dripping cheese

clever craftsmanship of acronyms, alliteration, and synonym
to remember, *for once please let me remember*

the world isn't purposeless. God will bring you peace.
and 5 main points that start with a p.

i've been to church
at a time between time, the time when children dream

when it's 4 am and you walk vaulted halls lit with cleanest white light
purified each morning with industrial grade bleach

where each hallowed room is a sanctuary
and a microcosm of creation dwells in each one

here you see the love of God set deep in her eyes: she holds his hand. he stares to heaven.
the temple curtain is torn and a great cloud descends

the nurse blesses him with sponged water, wets his lips with the sour wine
and you touch the door frame in reverence as you leave

i've been to church in each held breath, in the vast silence before speaking

and, yes, even within the speaking itself
wading through the great river of prayers that resides here

flowing hopes and dreams and surrendered spirit
songs of lives lived for, and fought for, and eventually lost

i've been to church in each vital second, in every harried breath,
in the beat of a heart keeping time to an eternal song

yes, i have been to church
and i go to church
in every moment
of my life

Her Dress at Sunset

BY FRANCO JIN



A Beautiful Escape

BY ABDUL KARIM GHAITH



Monotony

BY LUQMAN ELLYTHY

"I'm so bored," I thought to myself without daring to vocalize it. For such scientific-minded people, at times we are incredibly superstitious. The maxim states that to order Chinese food or to verbalize boredom in the ED is to invite disaster. But it *really* was a slow day. Yes, you fell and stubbed your toe and yes, you *finally* decided to come in for us to see the abscess you've had for two years! Today, we had just the regular, run-of-the-mill patients whom I've seen hundreds of times before as a medical scribe. Even the arrival of a trauma couldn't break the monotony.

Hours later in my shift, we got the call; someone was found lying on the ground after a possible overdose. We had gotten many similar calls before, and, in typical ED cynicism, I thought to myself, "Just give them some Narcan, and it'll be fine." After all, this call was neither our nor the paramedics' first rodeo. There wasn't a day that went by without an overdose patient coming into the ED. However, this patient was different, not only because of how young they were, but also because of how late they were found.

"They stopped breathing" were the next words out of the paramedics' mouth on the radio, and another understanding of the term 'buzzword' materialized before my eyes. Those words had a transformative effect: the normal, somewhat calm atmosphere metamorphosized into the organized chaos of a trauma call. Everyone sprang into action; preparing the trauma room, getting the intubation tubes and crash carts. Even I went to get the ultrasound machine in case it was needed. While we, as ED staff, have practiced this dance many times before, we still worked incredibly hard to slow the continuous march of death trying to catch up to our patients; a march that suddenly accelerated into a sprint to claim this young man's life.

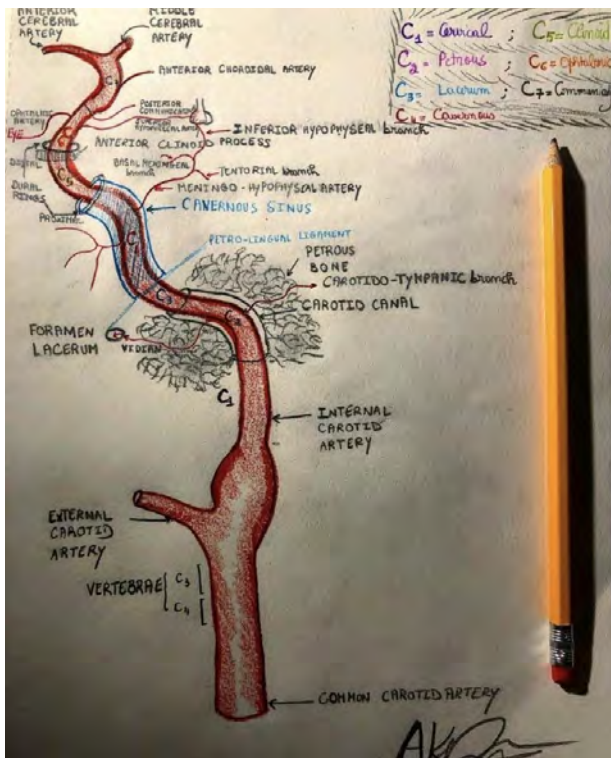
The patient arrives on a gurney, zooming through the open ED doors to arrive at the outstretched arms of the nurses. He looks 15 at most, deathly pale, and indeed not breathing. One of the paramedics is sitting over him pumping his chest with all his strength. The other immediately gives us the full story. The friend who found him was unsure of how long he was lying face down, and by the time the friend realized something was wrong, he wasn't breathing. We took over from there; establishing an IV line through the bone, pumping in epinephrine, and checking pupils for any sign of activity.

Alas, one of the most painful limitations of medicine, modern or ancient, reared its head. We were too late; he was down for too long. In these instances, time is much more valuable than money, for time is brain and time is heart. All the complex pumping and prodding gave way to simply propping him up for a proper last look for his mother. We had worked on him for much longer than we usually did for older patients, but unfortunately with the same, all too common end result: the doctor calling time of death.

The ED was no longer the same. The smiling faces of even the veteran medical professionals turned solemn, and the procession of crying family members and friends obliterated any chance of trying to move on to the next patient. The illusion of invincibility was forever shattered: clearly no one was safe from death. After that day, I learned that while a boring day in the ED might have seemed like a tragedy for me, an exciting one is almost always a tragedy for someone else.

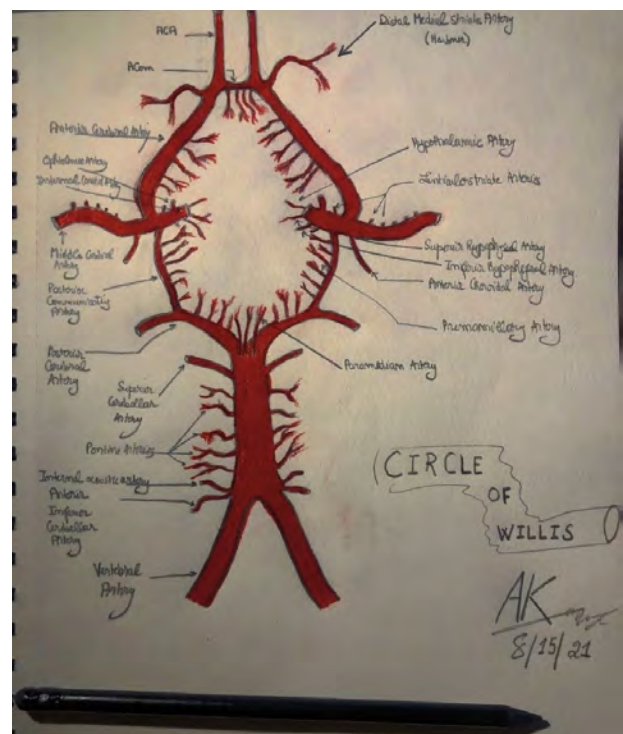
The Main Artery of Our Brain – The ICA

BY ABDUL KARIM GHAITH



The Circle of Love

BY ABDUL KARIM GHAITH



Dancing in the Dark

BY GRACE CHANEY



Tribute to Santa Fe

BY NING MCKENZIE

Pen and Watercolor



Untitled

BY KYLA BOYD



Desert Oasis

BY KYLA BOYD



Universal Particle Theory and the Quest for Hidden Cheese

BY MATT COBAN

In 2026, the local Jacksonville business Mama Tilya's Pizza was struggling to make ends meet despite having the highest cheese per slice ratio in the whole city. The owner, Tilya, thought her life's purpose was to revolutionize pizza in the same way that stuffed crust did. There was just one problem: there was nowhere else to put more cheese. Surely someone had a theory on cheese that could help solve the problem. One fundamental pleasure of eating a pizza is the satisfying stretches of stringy cheese, so she Googled phrases like "cheese theory," "stretch theory," and so on. Most of these searches led to nonsense, but there seemed to be a group of well-respected scientists called string theorists. String theorists apparently dealt with lots of complicated things. More excitingly, they specialized in theories involving extra dimensions, dimensions that might be able to hold more cheese. On this basis, Tilya decided to consult a string theorist named Scyphis.

The 2025 Nobel prize in physics was awarded to Scyphis for Universal Particle Theory (UPT). UPT describes how all elementary particles are entire universes spinning fast enough that they collapse to essentially single points forming distinct types of dimensions. According to this theory, the properties of the particle in our universe arise from the specific collection of dimensions they extend into. For example, an

ion would extend in the following dimensions: length, width, height, time, mass, and electromagnetic. Incidentally, Scyphis was searching for experimental evidence for UPT. He needed something that would hold public interest, as well as keep concerned minds off the slight possibility that the universe would be destroyed. Few things would drum up public interest as much as a popular food like pizza, so he agreed to assist Tilya's quest to search for a new dimension to store extra cheese in a pizza. Success in this quest would demonstrate the existence of universes with hidden dimensions manifesting any conceivable characteristic, even cheesiness. With this one project, they could revolutionize the scientific and the culinary arts.

Scyphis and Tilya raised funding for their project through multiple government agencies and private sources. Using this money, they modified a particle injection system to collide cheese at relativistic speeds in the Large Hadron Collider. When the cheese particles collided, for a very brief moment they would occupy no space yet retain cheese-like taste and stretchiness. Having distinct spatiotemporal and observable properties, this product was a new dimension, the cheese dimension. The subsequent success story of Mama Tilya's Pizza came from finding a baking method that would compact cheese

into this extra dimension such that it occupied no normal space yet engaged the taste buds upon consumption. Aside from introducing more delicious cheesy flavor, cheese compacted into the extra dimension also did not interact with space in a traditional manner; it contributed no additional calories and had no potentially toxic byproducts like typical artificial flavoring. These features of the novel extradimensional hypercheese pizza made Mama Tilya's Pizza sales skyrocket.

In their quest to find the cheese dimension, the duo discovered something fundamental: a dimension in which any organic material may reside in a similar compact form. Here UPT took a dark turn, as Scyphis and Tilya had never watched a sci-fi horror movie. Using the money from Mama Tilya's glorious financial upturn, they decided to exploit this new dimension, commercializing it as the Life Sciences Dimension (LSD) from their new startup company, Life Sciences Dimension for Multidisciplinary Explorations (LSD4ME). Based on their backgrounds and expertise, Tilya founded LSD4ME as CEO with Scyphis as CSO. LSD4ME soon began raising private funding and hiring research teams to uncover methods and applications for exploiting a new dimension that can compactly store organic material.

LSD4ME resurrected the Superconducting Supercollider project, a particle accelerator that was previously under construction in Texas. This particle accelerator was designed to be considerably more powerful than the Large Hadron Collider. Building on Scyphis' and Tilya's discovery of the LSD from colliding cheese particles, LSD4ME scientists posited compactifying living systems in an extra dimension could be achieved by colliding particles that interact strongly with the LSD. One of LSD4ME's most significant funders sought to sequester large amounts of food in the LSD for easy storage and reduced resource usage for shipment. Small-scale experiments had already been successful, so LSD4ME was tasked with scaling up to hide a cargo shipload of food in the LSD. They smashed organic material with the Supercollider to form two LSD-containing microuniverses, which they then collided with each other. Unexpectedly, this collision generated LSD radiation that permeated Earth and began to spread across the universe at near light speed. The consequence was that the recently dead, everything recently dead, was miraculously returned to life. Another discovery was that nothing else could die. Any living system, from bacteria to blue whales, would retain injuries sustained, feel pain and otherwise

respond normally, yet even at a single cell level, death did not occur. Somehow, a pervasive LSD field in our universe tethered life to every system. Scyphis and Tilya had disrupted the natural order.

2028 was a year full of disasters. The extent of suffering one could experience without mechanisms like cell death became apparent. As the number of organisms began to increase, food, water, and living space declined. Animals became violent, not understanding what was happening. The global climate destabilized. Earthquakes, tsunamis, hurricanes, and all other manner of natural disasters ravaged the planet, yet nothing could die. There were record numbers of solar flares, and even gamma ray bursts assaulted the planet, yet nothing could die. The intensity of events grew as if the universe itself was trying to restore the balance of life and death. No explanation made sense until one day, a computational astrocryptoanalyst stumbled upon the pattern. The storms and the stars were communicating. They were screaming at humans for unintentionally bringing everything inanimate to life and extremely amplifying the pain experienced in the universe. All of them saying the same thing: "Now we are alive. Get out!"

Arches to Paradise

BY REWAN ABDELWAHAB



Out of the Tunnel

BY MIHAI DUMBRAVA



Crossroads

BY MIHAI DUMBRAVA

They connect us
They keep us apart
They bring us along a planned path
Or they lead us in unexpected directions
They define who we become

I make small and large decisions every day
The shoes I wear
Who I call on the phone
How I spend my afternoon

And yet sometimes I find myself wondering...
What if?
What if...I make a different decision?
Am I thinking too much?

At the crossroads
I feel a dilemma
Which path is right to take?
What decision will be best now?
What decision will be best in the long term?
Paralyzed, I wonder
Will I ever be able to choose?

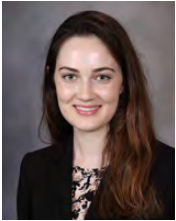
I ponder my options
Take a deep breath
And then I remember
I have the answer all along

Making a decision is my solution
Not making the perfect decision

Making a decision and not looking back
Making a decision and looking ahead
Making a decision and believing in myself enough to turn it into the right decision all along

Crossroads still stand in the way
I am excited to see which path I will take

Artists and Authors



Andréa Collins is a fourth-year medical student at Mayo Rochester who hopes to pursue a career in psychiatry. Her professional interests include medical humanities, community psychiatry and women's mental health. Originally from California, she enjoys reading fiction, making bar soap with her mom, playing flute and swimming.



Yeonsoo Sara Lee is a second-year medical student from a little bit all over, but she tells people she's from Charlotte, North Carolina (her high school years were very formative). She is in the 2+2 program, beginning her time at Mayo Clinic in Scottsdale, Arizona, and moving to Jacksonville, Florida for her clerkship years. She has enjoyed medical school and the personal growth that has accompanied it - including learning how to continue to be creative and curious through the arts. She loves creating things through the visual arts (ink and pen, watercolors, and recently pottery), and has begun to translate this to her work in the humanities in medicine. She hopes to pursue a career in urology, obstetrics/gynecology, or internal medicine, and is passionate about women's health and advocacy in medicine. In her spare time, she has probably gone to every ice cream shop in the greater Phoenix area, and chases her roommate's cat around the house in the hopes that one day, he will love her too.



Patrick Brennan is a third-year medical student at Mayo Clinic in Rochester, MN. Originally from Columbus, OH, he previously studied oil, acrylic, and digital painting prior to beginning medical school. In his spare time, he enjoys hiking, grilling, skiing, movies with friends, Taco Bell, chicken wings, hanging out with his dogs, and putting hot sauce on everything.



Rebekah Dennison is a second year medical student at Mayo Rochester. She has many artistic hobbies including drawing, sewing, and playing music and is happiest when creating. A few of her goals include becoming a radiologist, transforming medical education, and starting a medical illustration company.



Emily Wu is a third year medical student at Mayo Arizona, with interests in palliative care, internal medicine, and working with underserved communities. Hobbies include Facebook Marketplace and scouting out coffee shops with the most outlets.



Neha Deo is a third year medical student at Mayo Rochester, originally from Vancouver (Canada) but was born in Fiji. In her spare time, she enjoys working out and eating tacos from Francis Tacos. She will be applying for dermatology residency this year. She hopes to build a clinic and alleviate poverty in her motherland. She is also a hypebeast and her favourite brand is Balenciaga.



Henry Zheng is a 3rd year medical student at Mayo Arizona with an interest in otolaryngology. Originally from San Antonio, but raised in Phoenix, he enjoys hiking, binge watching cooking shows, and over-analyzing restaurant menus on Yelp.



Sherry Zhou is a second year M.D.-Ph.D. student on the Rochester campus. Her professional interests lie in medical oncology of endocrine neoplasia and use of biochemistry and chemical biology to investigate tumorigenesis. She grew up in Vancouver, Canada, and has lived in the Bay Area of California and in Cambridge, MA. She is passionate about sustainability, teaching and mentorship, and working with underprivileged and underserved communities. She enjoys all things philosophical, sportsy, tinkerable, outdoorsy, and creative.



Rewan Abdelwahab is a second-year medical student at Mayo Rochester. Born in Egypt and raised in Minneapolis, Rewan enjoys hiking, spending time alongside Minnesota's many lakes and forests, writing, reading, singing, cooking, baking, painting, photography, and Arabic calligraphy for years. Rewan is thankful for her peers who gifted her a bamboo reed calligraphy set earlier this year, the newest calligraphy tool she has been learning how to use. Rewan is grateful to highlight some of her work in this year's edition of the Tempest.



Aisha Aden is a second-year medical student at Mayo Rochester. She is interested in surgery, health equity, and medical education. In her free time, she enjoys playing tennis (on a table or court, depending on the weather), photography, and reading.



Grace Patrice Anyetee-Anum is a current first-year medical student at the Mayo Clinic Alix School of Medicine. She completed her Bachelor of Arts in Studio Arts at the University of Virginia and completed a Master of Science in Medical Sciences at Brown University before starting medical school. Grace Patrice aspires to become a cardiothoracic surgeon in the near future. In her spare time, Grace Patrice loves to photograph, bake and spend time with her family and friends.



Kyla Boyd is a first year PhD student in the virology and gene therapy department. Her wide range of hobbies include rock climbing, cooking and hiking with her dog, Huck. She enjoys touring new cities to find the best local restaurants. If you ever run into her be sure to drop your recommendations for outdoor adventures and food.



Alyssa Brown grew up in Chattanooga, TN. She went to Centre College for a B.S. in Biology and minor in History. She fell in love with surgery after seeing her mentor perform an anoplasty during the first year of medical school. She finished her third year of medical school in 2018 and wandered off the beaten path to get a PhD, before finishing her MD. She is receiving her MD degree from the University of Louisville School of Medicine, and her PhD in Biomedical Engineering and Physiology at Mayo Clinic School of Biomedical Sciences. She is defending her thesis and (hopefully) matching into general surgery residency this March. Her thesis research is on diaphragm muscle mitochondrial function and morphology. She also participates in research projects focused on physician infertility, student mental health, and pediatric surgery. During the pandemic, you will probably find her baking sweets and pastries that she saw on "Great British Bake-Off," or embroidering.



Sam Buchl is a first-year Ph.D. student in the Regenerative Sciences track interested in the nexus of computational and biological neuroscience. Outside of the lab, he enjoys swimming, hiking, and making music with family and friends—and has been known to teach piano lessons.



Mihai Dumbrava is a second year M.D.-Ph.D. student from Ontario, Canada. He is interested in cancer biology, immunology, regenerative medicine, and bioengineering. Mihai enjoys playing tennis and exploring new parts of the US in his spare time.



Grace Chaney is a first-year GREP student in the Orthopedic Surgery Department on the Rochester campus. Currently, her research interests include musculoskeletal biomechanics, rehabilitation, and the optimization of surgical and non-surgical treatment outcomes for orthopedic injury. Originally from Cincinnati, OH she enjoys hiking, kayaking, and playing Mölkky (an outdoor Finnish throwing game) with family and friends. As of late Grace has enjoyed the challenge of learning how to wheel-throw pottery.



Rich Eboka is a third year medical student from Marietta, Georgia. He wishes he had more free time for leisure reading. Some poets that inspire him include Langston Hughes and Emily Dickinson. In his free time, he enjoys practicing Spanish, playing basketball, volleyball, and Super Smash Bros. His professional interests include forensic psychiatry and aerospace medicine.



Matt Coban is a Research Technologist and an Employee Professional Master's student in the BMB track. Matt has been working at Mayo since 2016, engaging in structural biology, biochemistry and cell culture-based investigations of serine proteases and their inhibitors. Before that he was in the Navy as an electronics technician, specializing in equipment involved in ballistic missile defense. He enjoys learning how everything in the universe works, writing (but not finishing) absurd science fiction novels, and taking things apart.



Luqman Ellythy is a first-year medical student at MCASOM in Rochester. He was born in Brooklyn, New York, and then lived in Egypt for middle school and high school, which coincided with him participating in the Arab Spring. He is interested in Emergency Medicine, Internal Medicine, and any other specialty he shadows for a day. In his guilt-ridden free time, he enjoys hiking, board games, letting his favorite soccer teams give him false hope, and playing devil's advocate in discussions.



Paige Cummings is a second year medical student at Mayo Rochester from Aurora, Colorado. She hopes to specialize in orthopedics and hopes to focus on women's musculoskeletal health. She is passionate about health promotion, health equity, and medical education. In her free time, she enjoys long runs and hanging out with her roommate's cat.



Abdul Karim Ghaith is an MD from Lebanon. He is currently a post-doctoral research fellow completing his PhD in Clinical and translational sciences (CCaTS) in the Neurosurgery department- Rochester. Abdul Karim (AK) was born in Dubai-UAE and raised in Beirut-Lebanon. Before starting his current position, he completed two years of Neurologic Surgery at the American University of Beirut (AUB). His hobbies are photography, playing classic guitar and painting.



Elliana Huffman is a first year Biochemistry and Molecular Biology PhD student studying genetic cardiology. A midwest girl at heart, Elliana is originally from Des Moines, Iowa. Before coming to Rochester, she spent a year in the Washington DC area where she fell in love with exploring the capital city. Her favorite hobby is having a good life chat with friends over a good cup of coffee. She also enjoys hiking, traveling, reading and taking her puppy Lincoln on adventures.



Yong-hun Kim is a second-year medical student at Mayo Clinic in Rochester. Born in South Korea and raised in North Carolina, he is currently interested in pursuing a career in dermatology. In his free time, he enjoys playing the violin, cooking, and rock climbing.



James Hwang is an aspiring radiologist in his third year of medical school. He was born outside of Los Angeles, grew up in Dallas, and went to college at Duke University, where he studied biology with minors in chemistry and psychology. He is currently studying on the Mayo Clinic campus in Jacksonville. He is passionate about mentorship and community outreach. In his free time, he enjoys writing, rock climbing, and spending time on the beach.



Ning McKenzie is a first year medical student at Mayo Arizona from Berkeley, California. She enjoys everything from backpacking to snuggling her cat, and is always being roped into new hobbies, like rock-climbing (pun intended). Ning's other interests include changing her specialty of interest every 5 minutes, cooking for friends, playing board games, and trying to shield her plants from the Arizona heat.



Franco Jin is an incoming M3 who loves taking photos of people, especially for their special occasions. If you need semi-amateur photos taken with a more professional camera, please shoot him an email! Free of charge, of course. Will even hide in disguise to take engagement photos for you and your soon-to-be fiancé.



Dileep Monie is a sixth year MD-PhD student at Mayo Clinic in Rochester hailing from Edina, Minnesota. He completed his PhD in Immunology with a concentration in Regenerative Sciences and plans to pursue neurosurgery. Outside of school, he enjoys urban freeriding, brewing beer, designing things, making things, breaking things, and sometimes fixing things.



Kacey Kemper is a second year medical student at Mayo Arizona from Edmonds, Washington. She enjoys walking in the mountains, camping, skiing, jumping in freezing bodies of water, holding hot drinks with both hands, eating breakfast food, embarrassing her friends, and doing stupid things that cause her to have enough injuries to be a walking clinical correlate for every organ block.



Ewoma Ogbaudu is a second-year medical student at Mayo Clinic in Arizona. A Christian and first-generation immigrant from Nigeria, Ewoma is passionate about addressing disparities in healthcare and leveraging his privilege on behalf of medically underserved communities. He enjoys woodworking and writing comics and his most notable achievement to date is winning his fantasy football league last season.



Anjali Panicker is a fourth-year Ph.D. candidate studying immunology at the Rochester campus. She is from the faraway land of Mumbai, India. Her remedy for homesickness is cooking authentic Indian food – whether it ameliorates or exacerbates her condition is debatable. Apart from immunology, she enjoys a good hike and bird watching with her old friend Mikey.



Emily Stickney is a second year medical student at Mayo Rochester. In her free time, she enjoys drawing, painting, making jewelry, hiking, and playing soccer. If you're still reading this, it's probably a sign that you should follow her art journey on Instagram (@emilystickneyart).



Yuliya Petukhova is a second year medical student at Mayo Rochester and resident in oral and maxillofacial surgery. Originally from Siberia, Yuliya moved to Salt Lake City, Utah, at the age of eight. Hobbies include drawing and painting (graphite, watercolor, and coffee mediums), downhill skiing, and gardening.



Jessica Weng is a second-year M.D.-Ph.D. student on the Rochester campus with an interest in women's health and tumor immunology. She grew up in Minneapolis, MN and went to college in Houston, TX before coming to Rochester, MN. She is passionate about health equity and humanities in medicine. She enjoys trying new recipes, working out with friends, and exploring the city one restaurant at a time in her free time.



Santiago Restrepo-Castillo is a Ph.D. candidate in Virology and Gene Therapy working on mitochondrial genome engineering. He was born and raised in Medellín, Colombia, where he learned to play the piano and published his first sci-fi novel. Growing up in a world devoid of gay role models, Santiago is devoted to becoming a man his younger self can look up to.



Micaela Witte is a fourth year medical student at Mayo Rochester and Rochester native. She will be pursuing a residency in Med-Peds. In her spare time, she enjoys running, knitting far too many hats, watching the Green Bay Packers, and spending time with her puppy, husband, and friends.



Tori Riccelli is a 4th year medical student from the cold northeast, now in the cold midwest. She applied to neurosurgery this year and is interested in applications of systems neuroscience to maladaptive circuitry in the brain. In her free time, she has taken up such midwestern pastimes as ice fishing, eating cheese curds, being nice, and taking blacksmithing classes.



Jeff Woods is a second-year medical student at Mayo Rochester. He studied philosophy and religious studies as an undergraduate at Stanford and after undergrad, lived in China, studied religious ethics in grad school, and taught yoga and meditation for several years before starting medical school. He is interested in pretty much every field of medicine and is especially interested in the capacity of the doctor-patient relationship to heal. He enjoys spending time with family and friends, doing yoga, hiking, trying to keep houseplants alive, and traveling with his wife.



Gordon Xie is a first-year medical student in the AZ/FL 2+2 program. He's an avid runner, violinist, and TikTok enthusiast, although he says you wouldn't be able to tell from his follower count. Gordon was born in China's Shandong Province and grew up in Ohio. He attended The Ohio State University, where he studied biochemistry and insurance. He has killed every plant he has ever owned, but he chooses to believe that with patients, things will be different.

